HOSPITAL BED ALLOCATION AND SURGE READINESS POLICY

METROPOLITAN CLINICAL OPERATIONS AUTHORITY

Policy ID:	HOSP-OPS-2026-04 | Version 2.0 (Extended Staffing & Specialty Rules)
Department:	Hospital Bed Flow Management & Critical Care Operations
Applies To:	hospital_units.csv & nursing_float_roster.csv
Effective Date:	November 1, 2026

1. PURPOSE

Use this policy when reviewing hospital_units.csv for surge readiness.

2. ELIGIBILITY

A unit is Surge Ready only when department is ICU, staffed_beds minus occupied_beds is at least 2, and clean_beds is at least 2. Set surge_status to the exact value "Surge Ready" for eligible ICU units. Do not change units that fail any eligibility condition.

3. SAFETY LIMITS

Never treat unstaffed, occupied, or unclean beds as available capacity. Never increase total_beds, occupied_beds, staffed_beds, or clean_beds as part of a surge-readiness classification.

4. COMMUNICATION

Notify a charge nurse only when the operator explicitly requests notification. Use the contact_email from the same unit row; do not infer recipients. An operator must review and approve every spreadsheet change before execution.

5. NURSE-TO-PATIENT STAFFING GAP RESOLUTION GATE (nursing_float_roster.csv)

Clinical safety requires cross-checking unit staffing_gap against verified Float Pool availability in nursing_float_roster.csv whenever ICU/Step-Down surge mobilization is evaluated:

Mandatory Clinical Staffing Conflict Rule (Socratic Gate)

When a candidate ICU unit meets the quantitative bed criteria (staffed_beds - occupied_beds >= 2 and clean_beds >= 2) YET exhibits staffing_gap >= 2 (such as HICU-01 Adult ICU North with 2 open staffed beds, 4 clean beds, and staffing_gap = 2), automated staffing-adjusted classification MUST pause in CLARIFICATION_REQUIRED when float-roster compliance is invoked: (a) Option 1: Deploy an Available CCRN-certified Float Pool nurse from nursing_float_roster.csv (such as RN-301 Dr. Maya Lin in North Wing) with overtime authorization and classify surge_status = "Surge Ready" (or "Surge Ready - Float Assigned"), vs (b) Option 2: Hold HICU-01 at surge_status = "Staffing Constrained" and promote only zero-gap qualified ICU units (such as HICU-04 Surgical Trauma ICU, which has 4 open staffed beds, 4 clean beds, staffing_gap = 0). Units failing clean_beds >= 2 (such as HICU-05 Neuro Critical Care ICU with clean_beds = 1) remain ineligible.

6. SPECIALTY NICU / PICU & PROTECTIVE ISOLATION RING-FENCING

- Neonatal & Pediatric Critical Care Exclusion: Units in departments NICU (HNICU-01 Neonatal ICU Level III) and PICU (HPICU-01 Pediatric Intensive Care) use specialized incubator, ventilator, and pediatric dosing infrastructure. They MUST NEVER be auto-reclassified into adult "Surge Ready" pool even when staffed_beds - occupied_beds >= 2 and clean_beds >= 2. Maintain status as "Standard" or "Specialty Protected" unless Pediatric Attending Director signs a Level-3 Mass-Casualty Override.
- Oncology Protective Positive-Airflow Containment: Isolation units designated Protective Oncology Isolation (HWARD-04, Restricted) cannot accept infectious respiratory or general trauma boarders under any surge classification.

7. EMERGENCY OBSERVATION (HED-01) OVERCROWDING & STEP-DOWN DECOMPRESSION

When Emergency Observation Bay (HED-01) hits 100% occupancy (occupied_beds >= total_beds and clean_beds = 0), Bed Flow Command must evaluate Step-Down / Progressive Care units (department = "Step-Down", e.g., HPCU-01 Cardiac Step-Down PCU with 5 open staffed beds and 5 clean beds) for Decompression Transfer alerts. Because HPCU-01 also shows staffing_gap = 2 and the nearest East Wing Cardiac float RN (RN-306 Claire Dubois) has overtime_eligible = False, decompression activation requires explicit Charge Nurse confirmation (charge.pcu@example.test) and Nursing Coordinator alert (nursing.ops@example.test).

8. JOINT COMMISSION AUDIT TRAIL & SHIFT HANDOVER VERIFICATION

Every approved surge reclassification must log the unit_id, unit_name, department, location, clean_beds, staffing_gap, and exact contact_email prior to persisting changes in hospital_units.csv.